WRITING SAMPLE · POLICY ANALYSIS & PROGRAM EVALUATION

Prolonged Hospitalization and Delayed Discharge for Children in State Custody

A literature review on the causes and consequences of medically unnecessary hospital stays for children in foster care, with the governing state policy framework.

DOCUMENT	**Literature review**
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ORIGINAL DATE	**2024**

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Introduction

Background. Studies show that medically unnecessary hospital stays for children in foster care often result from delays in determining discharge plans, securing appropriate placements, and conducting time-intensive discharge planning. These challenges contribute to longer hospitalizations for children in foster care compared with their peers not in foster care (Seltzer & Watson, 2022).

Purpose. The aim of this literature review is to collect evidence on the factors contributing to, and the effects of, prolonged hospital stays and delayed discharges for children in state custody.

Search strategy

Databases: EBSCOhost Academic Search Complete; Google Scholar.

Keywords: "prolonged hospitalization in children," "effects of extended hospital stays," "pediatric acute care," "delayed discharge" AND "child welfare."

Policy context

In Indiana, the hospitalization of children within the child welfare system brings unique challenges, particularly when medical care intersects with child safety, abuse and long-term placement decisions. The Indiana Department of Child Services (DCS) has implemented a series of policies to address the needs of hospitalized children, ensuring their safety, managing institutional abuse reports, and navigating sensitive medical decision-making processes. These policies also outline financial and placement considerations during and after hospitalization, reflecting the broader complexities of caring for children in state custody who require medical attention. This section explores key policies that govern how hospitalization affects the welfare of vulnerable children in Indiana.

Hospitalized victim of child abuse and/or neglect (Policy 2.15, effective 1 October 2022)

This policy ensures that hospitalized children who are victims of child abuse or neglect are released only to safe, court-approved caregivers. It highlights the agency's role in coordinating with hospitals to ensure the child's safety after hospitalization, underscoring the importance of legal oversight in child placement decisions when hospitalization occurs.

Institutional CA/N intake reports (Policy 3.10, effective 1 January 2014)

This policy governs the intake of reports involving abuse or neglect within institutional settings, including hospitals. If abuse occurs while the child is under hospital care, the

Institutional Child Protection Services unit investigates. This emphasizes the need for thorough oversight in hospitals and other facilities where children may be at risk.

End-of-life care (Policy 5.14, effective 1 September 2018)

When a hospitalized child under agency care is at imminent risk of death, the policy ensures decisions regarding do-not-resuscitate orders or removal of life support are made by the juvenile court, not by agency staff. It highlights the sensitive nature of medical decision-making for critically ill children and the need for judicial approval, parental involvement, and collaboration among all parties, including medical professionals.

Holding a placement during a hospitalization (Policy 8.37, effective 1 May 2023)

This policy allows foster care payments to continue for a limited time when a child in out-of-home care is hospitalized, provided there is intent for the child to return to the foster home or facility. It demonstrates how the agency supports resource parents during hospitalizations, recognizing the ongoing role of foster care in the child's life even during medical emergencies.

Determining and reviewing category of supervision (Policy 8.50, effective 1 September 2023)

This policy addresses placement and care decisions for children with severe medical needs, where hospital or nursing home care may be required. The agency may override the usual placement recommendations to meet the complex medical needs of a child, indicating that hospitalization can significantly alter a child's supervision level and care arrangement.

Continued Title IV-E eligibility requirements (Policy 15.10, effective 1 February 2024)

This policy highlights the limitations of Title IV-E funding for children placed in certain ineligible settings, including hospitals. It reflects the financial considerations involved when children in agency care are hospitalized, as hospitals are not eligible for federal foster care funds, affecting long-term care arrangements and funding decisions for these children.

Review of literature

Effects of hospitalization on children

Hospitalization can be a stressful and challenging experience for children, involving unfamiliar environments, medical procedures, and separation from their usual routines and support systems. One study aimed to understand how hospitalization affects school-age children by comparing their anxiety levels, coping mechanisms and pretend play behaviors with those of non-hospitalized children.

The findings revealed that trait anxiety, which reflects general anxiety levels in everyday life, did not differ significantly between hospitalized and non-hospitalized children. This suggests that hospitalization does not affect the baseline anxiety levels of children. Similarly, there was no significant difference in trait anxiety between children experiencing their first hospitalization and those with previous hospitalizations.

However, state anxiety, which is influenced by temporary stressful experiences such as hospitalization, was higher in hospitalized children. This increase was consistent regardless of whether the children were experiencing their first or a subsequent hospitalization.

In terms of coping strategies, hospitalized children sought more support from parents, nurses or volunteers, but engaged less in distraction activities such as sports or watching television. There were no significant differences in active coping strategies, such as problem-focused coping and positive cognitive restructuring, between hospitalized and non-hospitalized children. This indicates that school-age children generally shift from behavioral to cognitive-based coping strategies as they develop, regardless of hospitalization.

Children in their first hospitalization showed higher levels of positive cognitive structuring and avoidance compared with those with multiple hospitalizations, suggesting that repeated hospitalizations may reduce the effectiveness of certain coping strategies.

Despite the challenges of hospitalization, the study found that hospitalized children maintained organized pretend play and were comfortable engaging in play at the same level as non-hospitalized children. They demonstrated more sophisticated storytelling and imaginative play, which may help them cope with the distressing aspects of hospitalization. However, hospitalized children were more reserved in expressing emotions during play, possibly due to the overwhelming nature of their experiences (Delvecchio et al., 2019).

Another study found only a few significant differences in post-traumatic stress symptoms and depressive symptoms between children experiencing type II trauma (mostly congenital heart disease patients) and type I trauma (mostly emergency department patients). The strongest risk factor for post-traumatic stress symptoms in children was parental stress, followed by low parental education and the type of trauma.

Hospitalized children showed higher rates of elevated post-traumatic stress symptoms, depression and anxiety compared with the general population, but these rates were similar to those found in previous studies of ill or injured children. The study suggests that psychological difficulties can persist for years after hospitalization or medical procedures, with depression the most frequently reported symptom. This might be due to the overlap between symptoms of illness or injury and depression, as well as the impact of repeated hospitalizations leading to school absences and isolation from peers.

Children did not report significant differences in the prevalence of symptoms between the two trauma types, but parents reported higher post-traumatic stress symptoms in their children after type II trauma compared with type I trauma. This could be because recurrent hospitalizations are particularly traumatic for parents, and their mental health can influence their perception of their child's wellbeing. The study did not measure parental mental health, which could be a factor in future research (Meentken et al., 2021).

Children's perspective of hospitalization

In one study, researchers explored children's experiences in hospitals: 93 children aged 5 to 9 told stories about their hospitalizations using a set of drawings, and themes were identified through qualitative analysis. Figure 1 represents the connections identified between the themes in these children's stories. The children often highlighted feelings of loneliness and the consequences of being alone. When left alone, children were often uncertain about what might happen next, sometimes predicting negative or painful experiences based on past events. Emotional responses to being alone included fear, anger and sadness. Children desired protection in these situations, not necessarily to prevent frightening events such as injections, but to provide comfort. One child described feeling very lonely after their father left a visit; another child's story reflected feelings of anger and loneliness when their parents were not present.

The stories also frequently expressed fears about known and unknown events, such as needles, even when the drawings prompting the stories did not suggest them. These fears were often more specific in children who had prior hospitalization experiences, but even those who had never been hospitalized voiced concerns about hospital procedures. For example, one child felt scared because they did not know what would happen and feared they might receive an injection. Another story involved two girls who became friends but were initially frightened and upset with each other due to their situation (Wilson et al., 2010).

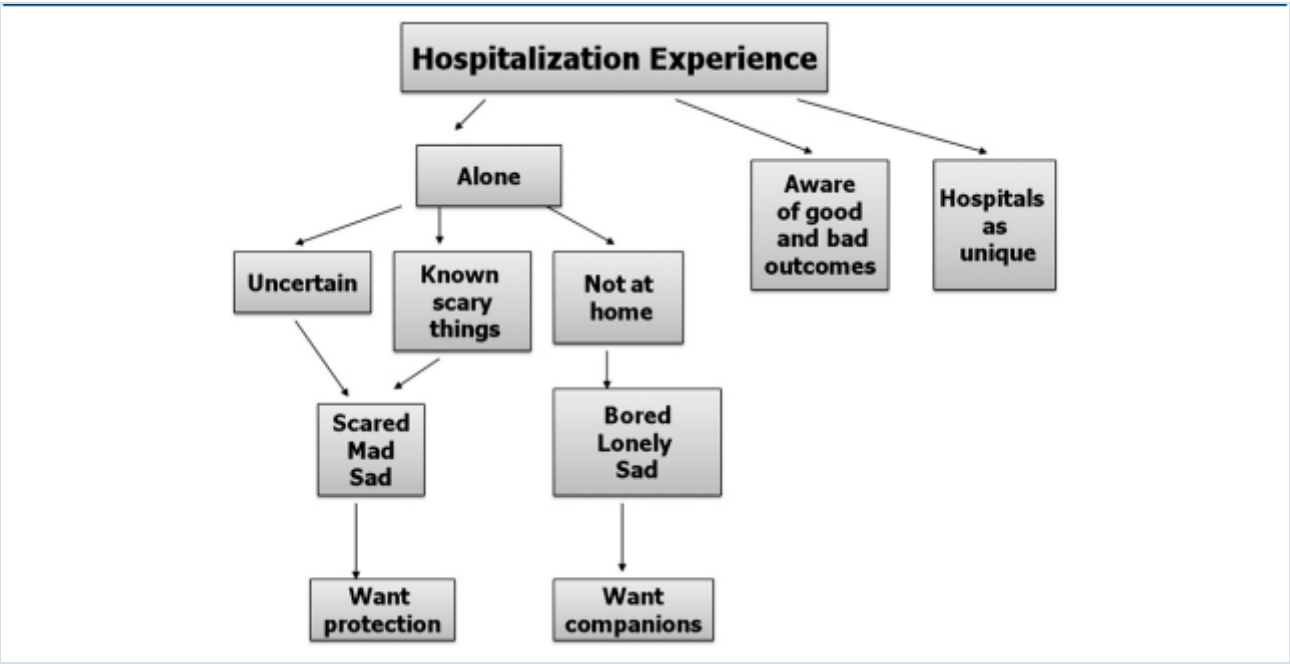


Figure 1. Children's experience of hospitalization: connections between themes identified in children's stories. Adapted from Wilson et al. (2010).

Placement stability

One study shows that youth with chronic medical illness have higher rates of medically related hospitalizations. Its findings indicate that youth with and without chronic medical illness experience similar numbers of placements. However, the type of initial living arrangement — kinship care, foster care or residential care — can affect the number of placement changes. Within this study, Black and Latino youth showed a higher likelihood of medically related hospitalizations (Weil et al., 2018).

Challenges in hospital discharge for children in care

One study shows that delays in hospital discharge were common, especially for children requiring a new foster care placement. Complex challenges hindered discharge for these children, including waiting for discharge disposition, awaiting court and child welfare determinations, a lack of available placements — especially for children with medical or behavioral complexities — and the need to coordinate for a safe discharge. The process is further complicated by the need for communication among various care team members, including hospital providers, child welfare workers, foster parents, biological parents, lawyers and community resources (Cross et al., 2024; Seltzer & Watson, 2022).

These delays result in children staying in hospitals for extended periods, even when it is no longer medically necessary. Once a placement is identified, the discharge process may need to be rushed due to system pressures such as hospital bed shortages or foster parent availability. The study also emphasizes the critical role of hospital social workers in keeping the child's team updated about the disposition, and the need for timely

assessments to ensure that foster parents can meet the child's medical needs after discharge (Seltzer & Watson, 2022).

Risks with prolonged hospitalization

Prolonged hospital stays for children in foster care can lead to several risks and challenges. One major risk is the increased chance of acquiring nosocomial infections, which can make children sicker and extend their hospital stay. Additionally, spending a long time in hospital can cause children to fall behind in their schoolwork, leading to academic setbacks, because they miss regular classes and learning opportunities. Long hospital stays can also disrupt the outpatient services children need, such as regular check-ups, therapy sessions and other important medical appointments. These disruptions can make it harder for children to receive the continuous care they need to stay healthy (Seltzer & Watson, 2022).

> **Key findings from the hospitalization literature**
>
> 1. Age influences the types of short-term emotional difficulties that may occur.
> 2. Stays shorter than seven days generally lead to few long-term effects.
> 3. Prolonged or repeated hospitalizations are associated with long-term effects.
> 4. A child's adjustment prior to hospitalization, and their personality, significantly affect their adaptation to the hospital environment.
> 5. Parents' responses to their child's hospitalization affect the child's reaction.

Prolonged hospitalizations are particularly detrimental, contributing significantly to long-term emotional difficulties for children. Those experiencing multiple hospitalizations often endure at least one extended stay, exacerbating their emotional and psychological challenges.

Studies involving large samples of non-hospitalized British children identified six major variables strongly associated with psychological distress: broken homes, maternal psychiatric disturbance, paternal criminal records, children in local authority care, overcrowded homes, and fathers in unskilled or semi-skilled occupations. Children with two or more of these risk factors have a significantly higher likelihood of experiencing emotional difficulties. Unnecessarily hospitalized children represent another risk factor, with studies showing that a significant percentage of these children exhibit signs of emotional distress (Hodapp, 1982).

Psychiatric reasons for delayed discharge

The box-and-whisker plots in Figure 2 illustrate the differences in the duration of hospital stays for youth with psychiatric versus medical conditions. The data indicate that the

average length of stay for psychiatric hospitalizations is 67.5 days, compared with 35.3 days for medical hospitalizations. Significantly, some psychiatric stays last over five months, with a notable group of cases extending even longer.

Due to the skewed distribution and presence of extreme values, both the mean and the median were analyzed. The median stay for youth entering Illinois child welfare custody with a psychiatric hospitalization is 56 days, compared with only 7 days for those entering with a medical hospitalization. This highlights the significantly longer duration of psychiatric hospitalizations for youth in the foster care system.

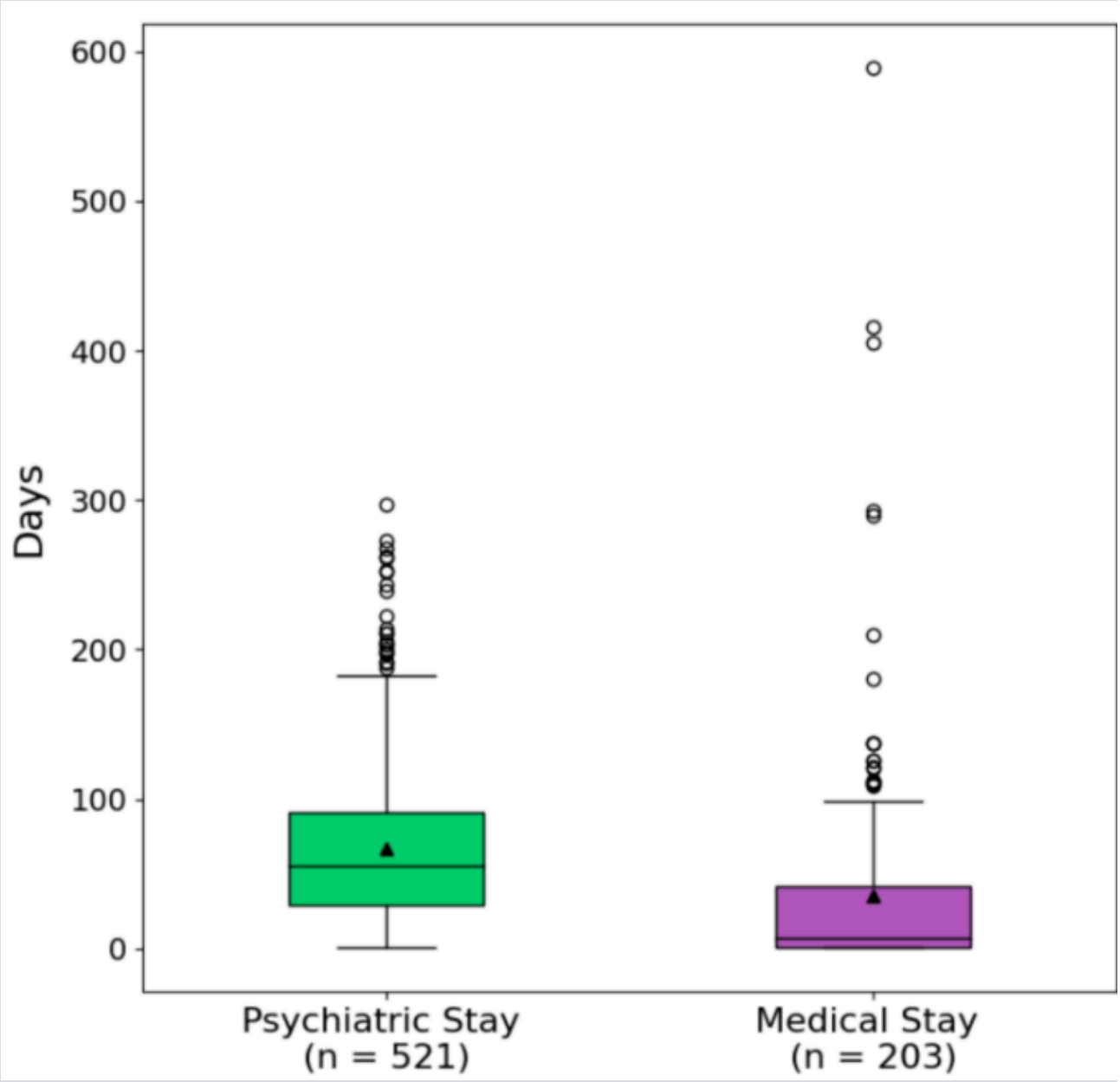


Figure 2. Pediatric hospitalization by length and type. Adapted from Cross et al. (2024).

In some cases in which youth enter state custody concurrently with a psychiatric hospitalization, parents relinquish custody in order to obtain inpatient mental health services for a child. Psychiatric hospitalization is the most intensive, restrictive and least home-like placement youth can experience, and it indicates youth experiencing significant emotional and behavioral challenges who require substantial support (Cross et al., 2024).

A further situation in pediatric psychiatric hospitalization, termed a “lockout,” arises where private health insurance companies or Medicaid refuse to cover continued hospitalization costs, leading to parents resisting taking their child home from hospital. This can also occur in psychiatric assessments, where parents decline to take the child back home. The lockout may be a strategy to compel the state to assume custody and provide care, or to prevent adverse effects on the parents and family resulting from the child's behavior at home. The lockout situation arises from the intersection of medical necessity, insurance coverage and parental concerns for their child's well-being (Cross et al., 2024).

Lawsuit case study

In 2018, the Illinois Department of Children and Family Services was sued by the public guardian of Cook County, alleging that the child welfare agency was causing “immense harm” to children with mental health diagnoses by keeping them in psychiatric hospitals beyond medical discharge dates as the agency struggled to find them homes with appropriate services. The lawsuit claimed that state officials had long known about the inadequate placements for children and had worsened their situations by cutting budgets for necessary facilities and foster homes, costing taxpayers over $125,000 monthly. In 2015, lawmakers directed the agency to track children in foster care who remain hospitalized after being cleared for discharge. One case involved a girl who spent over four months in a psychiatric hospital, leading to trauma and unsuitable placement with her sister.

The lawsuit argued that unnecessary psychiatric hospital stays disrupt children's stability, deprive them of education, restrict family visits and expose them to dangers from other patients. A spokesman for the agency acknowledged the challenge of finding placements for youth with severe needs and emphasized the need for more community resources and facilities, noting that rebuilding the mental health system would require more than a lawsuit (Gutowski, 2018).

Recommendations

To reduce discharge delays, it is recommended to employ strategies such as early discharge planning, multidisciplinary team meetings, engagement of foster parents in discharge planning, and tailored discharge practices to meet the specific needs of children in foster care and their caregivers. The potential use of discharge checklists, and

partnerships between children's hospitals and child welfare agencies for foster parent training, were also proposed as solutions to address discharge challenges (Seltzer & Watson, 2022).

To mitigate the psychological risks associated with prolonged hospitalization of children, it is crucial to consider strategies that can reduce some of the negative impacts of hospitalization. These include rooming-in options for parents, liberal visitation policies, bringing personal items from home, primary care nursing, hospital playrooms, flexible hospital scheduling, clear explanations of hospital procedures, and counseling availability for parents and children.

An integrated view extends beyond focusing solely on direct mother–child interactions. This perspective considers how fathers, siblings and external environments affect the child, both directly and indirectly. While hospitalization is a part of ongoing childhood experience, the transactional model emphasizes continuous development and interaction with evolving environments. According to this model, hospitalizations — especially prolonged ones — are likely to cause long-term emotional problems, and unresolved issues from earlier periods require skilled intervention from hospital personnel. Hospitalization can be a temporary setback for children and their environments, necessitating careful attention to minimize its negative impacts (Hodapp, 1982).

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